

West Midlands Upper GI EAG Recommended 2WW Form_FINAL_v1

URGENT REFERRAL: SUSPECTED UPPER GI CANCER

PATIENT DETAILS		REFERRING GP DETAILS
Surname: Surname	Forename: Given Name	Name: Free Text Prompt
Address: Home Full Address (single line)		Address: Organisation Name Organisation Full Address (single line)
NHS Number: NHS Number	Date of Birth: Date of Birth	
Mobile Tel No: Patient Mobile Telephone	Home Tel No: Patient Home Telephone	
Is an Interpreter required? ☐ Yes ☐ No If yes please specify the patients preferred language: Main Language		Contact Tel No: Organisation Telephone Number
Smoking status: Yes ☐ No ☐ Ex-Smoker ☐		*Referral date: Short date letter merged
BMI: Single Code Entry: Body mass index	Weight: Single Code Entry: Body weight	
Is the patient diabetic: Yes ☐ ☐ Insulin dependent No ☐ ☐ Non-insulin dependent		
Patient on Anticoagulation/antiplatelet medication? ☐ Yes ☐ No Please specify indication:		
☐ Warfarin ☐ Apixaban ☐ Rivaroxaban ☐ Aspirin ☐ Clopidogrel ☐ Other		
Patient has capacity to consent? ☐ Yes ☐ No		

*I have informed the patient this is a suspected cancer referral	☐
*I have emphasised to the patient the importance of being available over next 14 days for an urgent appointment	☐
*I have given or sent the patient a copy of the '2WW Patient Information Leaflet'	☐

*CLINICAL HISTORY / INFORMATION AND RECENT TEST RESULTS:
Please include detailed history of presenting complaint and relevant medical history, co-morbidities, current medication, allergies and/or any other recent investigations.
Relevant medical history, current medication and allergies please see below

REASON FOR REFERRAL – DYSPHAGIA – FOR ENDOSCOPY																																																			
☐	Dysphagia - clear description of food sticking following initiation of swallow (<i>please complete the Edinburgh Dysphagia Score below for all dysphagia referrals</i>). This is recommended by the BSG to help endoscopy units triage endoscopy referrals to the most likely to have a cancer diagnosis. <u>Please tick each individual item in the score</u>																																																		
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☐ 90-99	9	☐ No	0	☐ No	0
A	B	C	D	E	F
EDS = A+B+C+D+E+F EDS score:					
An Edinburgh Dysphagia Score of ≥ 3.5 will proceed to urgent endoscopy if fit.					
An Edinburgh Dysphagia Score of < 3.5 will be listed for endoscopy on a non-urgent basis or may be triaged for further telephone assessment and non-endoscopic investigations such as barium or CT imaging.					

REASON FOR REFERRAL – DYSPEPSIA – FOR ENDOSCOPY

☐	Age >55 years AND weight loss ($> 3\text{kg}$) AND any of the NEW following: - Upper abdominal pain - Dyspepsia (if dyspepsia without weight loss to refer to NICE CG184: GORD and dyspepsia in adults) - Reflux
	Please specify weight loss and duration

REASON FOR REFERRAL – JAUNDICE OR MASS FOR IMAGING

☐	Jaundice and ≥ 40 years
☐	Upper abdominal mass
*Please ensure the following blood tests have been undertaken (within 4 weeks) or <u>requested</u> prior to referral.	
☐	FBC ☐ eGFR ☐ Clotting ☐ U&Es ☐ LFTs

REASON FOR REFERRAL – SUSPECTED CANCER ON IMAGING – FOR MDT DISCUSSION

☐	Imaging report suggestive of gastric or oesophageal malignancy
☐	Imaging report suggestive of liver/ pancreatic/ gallbladder malignancy
*If an USS or CT has already been undertaken, please include report if available. If not, please request 2ww CT (Abdomen and Pelvis with contrast) on referral.	

*WHO PERFORMANCE STATUS - PLEASE CONFIRM PATIENTS PERFORMANCE STATUS AT TIME OF REFERRAL

☐ 0	Fully active, able to carry on all pre-disease performance without restriction
☐ 1	Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature, e.g. light housework, office work
☐ 2	Ambulatory and capable of all self-care but unable to carry out any work activities up and about more than 50% of waking hours
☐ 3	Capable of only limited self-care, confined to bed or chair more than 50% of waking hours
☐ 4	Completely disabled. Cannot carry on any self-care. Totally confined to bed or chair

Problems
Medication
Allergies